

Description

Syndromic surveillance is public health surveillance that emphasizes the use of near "real-time" health data and statistical tools for disease or hazardous event detection, situation awareness for mass gatherings and public health emergencies, and ad hoc and population health trend analyses. For syndromic surveillance purposes, required data on all clinical encounters must be sent to the jurisdictional public health agency within 12 hours of any given encounter, and in accordance with local or state laws and practices.

The Hurricane Impact Case provides an example of a clinical encounter that could take place in a freestanding urgent care setting.

In this test case, an adult male seeks care for gastrointestinal symptoms. He had been exposed to flood waters following a recent hurricane, and is experiencing vomiting, diarrhea, nausea, and abdominal pain. He is diagnosed with dehydration and treated with IV fluids, and given an injectable antiemetic to help with vomiting. The clinician determines that further investigation and treatment are better handled in an Emergency Room setting, and refers the patient for Emergency care. The jurisdiction requires that the syndromic surveillance messages are sent in with identifiable demographics. Syndromic surveillance data about the visit are transmitted to the state health department. This Test Case will test using SNOMED-CT Dx codes and full, legal patient name. This test case includes documentation of travel and addition of a final diagnosis post-discharge.

This test case involves 3 steps:

- a registration message (ADT^A04),
- followed by an update message (ADT^A08),
- followed by a discharge message (ADT^A03).

Messages in this test case will be used in a Manual Test Step under functional requirement testing for validating the HIT capability of populating Visit ID (PV1-19.1) and Visit Time (PV1-44.1) each with the same value for all messages included in this Test Case.

Comments

No Comments

Pre-condition

No Pre-Condition

Post-Condition

No Post-Condition

Test Objectives

This test case examines a Health IT Module's ability to create an ADT^A04-Registration message, an ADT^A08-Update message, and an ADT^A03-Discharge message within HL7 Version 2.5.1 Implementation Guide: Syndromic Surveillance, Release 1 - US Realm Standard for Trial Use July 2019, conformance requirements.

The following test objectives and variants are supported by this test case:

- Setting is Urgent Care.
- Chief complaint is captured as a narrative in PV2-3 Admit Reason.
- **Patient name is identifiable in the submissions:** Per ASTP/ONC, for certification testing HIT developers must demonstrate that their system supports the Patient Name and pseudonymized data in PID-5 (this policy is conformant to the named standard HL7 Version 2.5.1 Implementation Guide: Syndromic Surveillance, Release 1 - US Realm Standard for Trial Use July 2019). The patient name presented in an identifiable manner is required for this Test Case.
- **Diagnoses are submitted test using either ICD-10-CM or SNOMED-CT:** Per ASTP/ONC, for certification testing a Health IT Module can use either the ICD-10-CM or SNOMED CT® value sets in the submitted messages for all of the test steps in the Syndromic Surveillance Test Suite. Where the tool does not have test data that supports the Health IT Module's value set (either ICD-10-CM or SNOMED CT®), the developer of the Health IT Module must provide the codes and testers must perform a visual inspection of the messages for these test steps to ensure that equivalent and valid ICD-10-CM or SNOMED CT® are used to populate the messages. This policy is conformant to the named standard HL7 Version 2.5.1 Implementation Guide: Syndromic Surveillance, Release 1 - US Realm Standard for Trial Use July 2019). SNOMED-CT® codes are provided/required in the Test Data for coding the diagnoses in this Test Case.
- Adult case where age is measured in years.
- Addition of Height and Weight
- Addition of Diagnoses
- Update Diagnosis Post-Discharge
- Insurance is unknown

Evaluation Criteria

No Evaluation Criteria

Notes

None